

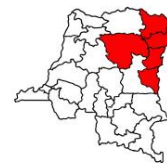


Ministry of Public Health, Hygiene and Social Welfare

National Institute of Public Health

Public Health Emergency Operations Center

MVE17 Incident Management System



Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°087/MVEBDB/09/08/2026

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**HIGHLIGHTS** (last 24 hours)

Over the past 24 hours, 87 new confirmed cases and 51 deaths (34 new deaths and 17 within CTE) have been reported. Ebola Virus Disease (EVD) cases have been recorded in the provinces of Ituri (57 cases), North Kivu (15 cases) and Haut-Uélé (15 cases) reflecting the continuation of active transmission.

ACCUMULATIVE CASE 4,381	ACCUMULATIVE CONFIRMED DEATHS 2011	LETHALITY 45.9%	PATIENTS IN ISOLATION / CTE 704	ACCUMULATIVE HEALED 869	TRACKING RATES CONTACTS 86.7% (Of the Day)
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Affected Provinces and Affected Health Zones

**5 Provinces Affected**

Haut-Uélé, Ituri, North Kivu,
South Kivu & Tshopo

**53 Health Zones**

Affected Out of 140
(37.8%)

**246 Health Areas touched**

Out of 3104 (7.9%)

Summary of key points from the last 24 hours

- In the past 24 hours, 87 new confirmed cases and 51 deaths (34 new deaths and 17 intra-ETC) of Ebola Virus Disease (EVD) were recorded in the provinces of Ituri (57 cases), North Kivu (15 cases) and Haut-Uélé (15 cases), reflecting the continuation of active transmission, mainly in Ituri.
- To date, the Democratic Republic of Congo has recorded 4,381 confirmed cases, including 2,011 deaths, representing a case fatality rate of 45.9%. Ituri remains the epicenter of the epidemic, accounting for 85.8% of cumulative confirmed cases and 65.5% of new cases reported in the last 24 hours.
- 20 patients were declared cured after obtaining two negative results for Ebola Virus Disease (EVD) (17 in Ituri and 3 in North Kivu). However, 17 deaths within Ebola Treatment Centers were recorded in Ituri (12) and North Kivu (5).
- The epidemic affects 53 of the 140 health zones in the five affected provinces. Ituri has 28 out of 36 affected health zones, followed by North Kivu (12/34), Haut-Uélé (6/13), Tshopo (6/23) and South Kivu (1/34).

Distribution of confirmed cases and deaths by affected province

(As of August 9, 2026)

Province	Case confirmed	Death (confirmed)	Lethality Health Zones touched	New cases confirmed (24h)
Ituri	3,761	1,620	43.1% 28/36 (77.8%)	57
North Kivu	495	338	68.3% 12/34 (35.3%)	15
Haut-Uélé	113	47	41.6% 6/13 (46.1%)	15
Tshopo	9	5	55.6% 6/23 (26.0%)	0
South Kivu	3	1	33.3% 1/34 (2.9%)	0
Total	4,381	2011	45.9% 53/140 (37.8%)	87

Confirmed cases and deaths by province and health zone (situation as of August 9, 2026)

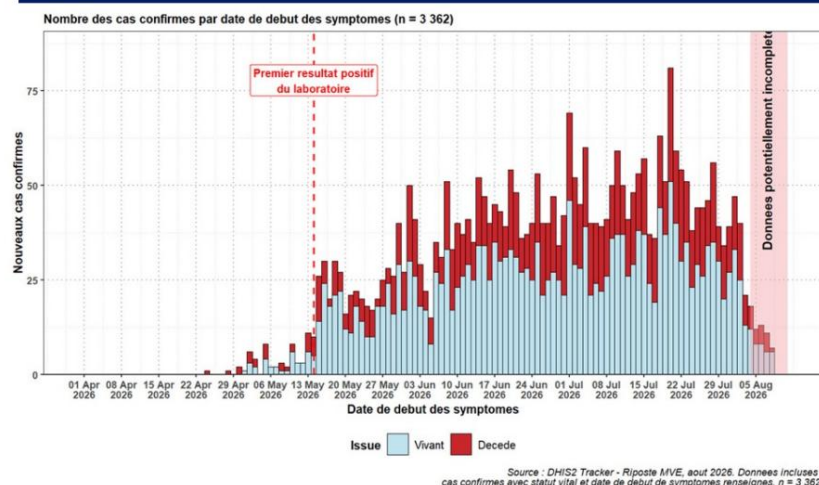
Province	Health Zone	New cases confirmed	Including deaths among the newly confirmed	Lethality	Death communities	Death intra-CTE/CT	Total death of the day
Ituri	Bunia	12	4	33.3%	4	ND	ND
	Nizi	9	1	11.1%	1	ND	ND
	Fataki	9	5	55.6%	5	ND	ND
	Rwampara	6	2	33.3%	2	ND	ND
	Lita	6	6	100.0%	6	ND	ND
	Mangala	6	5	83.3%	5	ND	ND
	Nia-Nia	4	1	25.0%	1	ND	ND
	Tchomia	2	0	0.0%	0	ND	ND
	Aru	1	0	0.0%	0	ND	ND
	Mongbwalu	1	0	0.0%	0	ND	ND
	Nyankunde	1	0	0.0%	0	ND	ND
	Bambu-mine	0	0	0.0%	0	ND	ND
	Mandima	0	0	0.0%	0	ND	ND
	Mambasa	0	0	0.0%	0	ND	ND
	Komanda	0	0	0.0%	0	ND	ND
	Logo	0	0	0.0%	0	ND	ND
	Lolwa	0	0	0.0%	0	ND	ND
	Drodro	0	0	0.0%	0	ND	ND
Ituri subtotal		57	24	42.1%	24	12	36
North Kivu	Blessed	6	4	66.7%	4	1	5
	Katwa	5	1	20.0%	1	0	1
	Butembo	2	2	100.0%	2	1	3
	Kalunguta	1	0	0.0%	0	0	0
	Masereka	1	0	0.0%	0	0	0
	Musienene	0	0	0.0%	0	3	3
North Kivu subtotal		15	7	46.7%	7	5	12
Haut-Uélé	Wamba	14	2	14.3%	2	0	2
	Isiro	1	1	100.0%	1	0	1
	Pawa	0	0	0.0%	0	0	0
	Boma Mangbetu	0	0	0.0%	0	0	0
	Rungu	0	0	0.0%	0	0	0
	Gombari	0	0	0.0%	0	0	0
Subtotal Haut-Uélé		15	3	20.0%	3	0	3
TOTAL NATIONAL		87	34	39.1%*	34	17	51

*Lethality was calculated solely on the basis of community deaths (n=34).

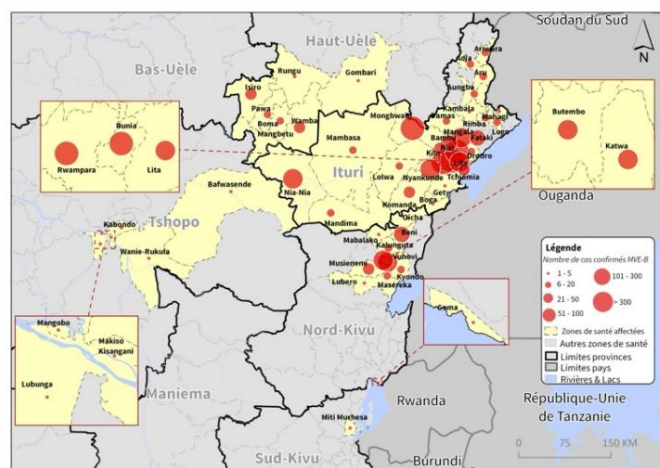
Status of alerts notified by province, as of August 9, 2026

DPS	New Alerts Received		Total alerts	Alerts verified and validated (suspected case)		Alerts verified and invalidated (no suspected cases)		Number of suspects investigated	Number of suspects investigated and transferred to CT/CTE (of the day)
	Living	Deceased		Living	Deceased	Living	Deceased		
Haut-Uélé	6	3	9	5	1	1	2	6	4
Ituri	650	58	708	68	57	563	1	125	60
North Kivu	596	61	657	52	51	544	10	103	52
South Kivu	8	0	8	1	0	7	0	1	1
Tshopo	44	0	44	7	0	33	0	7	7
Grand Total	1304	122	1,426	133	109	1148	13	242	124

Evolution of cases by date of symptom onset over the last 21 days



MAPPING OF CASES over the last 21 days



1. RESPONSE ACTIONS BY PILLAR

1.1. Coordination

1.1.1. Main Actions

- **North Kivu** : Support for the Beni zonal coordination and continuation of the mission support for the Butembo health zone.

1.2. Epidemiological surveillance

1.2.1. Main Actions

- In total, 1,426 alerts were recorded and verified in the five affected provinces for the day of August 9, 2026, of which 242 (16.9%) were validated as suspected cases and 242 (100%) were investigated and 124 suspects transferred to CT/CTE.
- The proportion of contacts followed up in the last 24 hours is at 86.7% (14,029 seen out of 16,183 to be followed up), above the threshold of 85%, with lower performance in Haut-Uélé (57.9%) linked to the completeness (66.7%) of the data.

1.2.2. Challenges

Low reporting of alerts and listing and monitoring of contacts around confirmed cases.

1.3. Update on checkpoint and point of entry (PoC/PoE) activities

1.3.1. Main actions

- Twenty alerts were detected at the different PoE/PoC (10 in Haut-Uélé, 6 in Ituri and 4 in Nord-Kivu): the live alerts (n=4) in Ituri were validated, isolated and referred to the care structures and the two lifeless bodies intercepted were swabbed; in Nord-Kivu, the two lifeless bodies were directed to the morgue for sampling and the two live alerts were invalidated after investigation; none of those in Haut-Uélé were validated.
- In total, 106,546 travelers were screened (92,432 in Ituri, 14,114 in Haut-Uélé), with a screening rate of 98%. In Ituri, only one of the 11 strategic PoE/PoCs (9.1%) operated without interruption.

1.3.2. Challenges

- Insufficient operationalization of PoE/PoC: PoE and PoC functionality remains low in Haut-Uélé, with only 10 out of 38 (26.3%) operational. • Constraints affecting PoE/PoC functionality: Performance continues to be hampered by shortages of supplies, PPE, and water in Haut-Uélé, non-payment of service providers, and non-compliance with health protocols by some authorities at the Mukulia PoC (North Kivu). These constraints reduce the effectiveness of health control, early detection, and surveillance at borders and along major transportation routes.

1.4. Laboratory

1.4.1. Main actions

- **405 samples** were analyzed, confirming **87 new cases** of EVD (positivity rate: **21.5%**).
- Ituri recorded **57 positive results**, including 24 deaths, while North Kivu and Haut-Uélé confirmed **15 cases each, with 7 and 3 deaths respectively**.

1.5. Infection Prevention and Control (IPC/EDS)

1.5.1. Main actions

- In **Ituri**, 20 rings were opened (52 in total), 47 sites identified and 47 decontaminated (100%); 76 EDS carried out on 85 death alerts (89.4%) and 77 bodies collected post-mortem, for a completeness of 8 out of 10 ZS (80%).
- In **North Kivu**, 45 health facilities were supported and 168 providers were briefed; 2 facilities were assessed by scorecard, 52 EDS were carried out and 47 bodies were collected and secured, for a completeness of 6 out of 12 health zones. • Tshopo did not transmit any PCI/EDS data.

1.5.2. Challenges

- Weak implementation of IPC standards: partial decontamination of households of cases in Ituri (40 out of 86), limited reporting completeness in North Kivu (6 health zones out of 12) and low card scores (37.1% in Lubero and 48.3% in Musienene), demonstrating a continued risk of healthcare-associated infections.

1.6. Continuity of care

1.6.1. Main actions

1.6.2.

- In **Ituri**, 30 ambulances were assigned to the response, of which 26 were available; 34 trips were made, 32 of which were successful, for 36 patients evacuated. The Ebola Treatment Centers (ETCs) recorded 82 admissions and 75 discharges, including 17 recoveries; occupancy reached 508 patients out of 840 beds (61%).

specifically in Ituri, with saturation of the Rwampara, ISTM and Nizi CTEs and the Bambu CT.

- **In North Kivu**, there have been 232 cumulative admissions to CTE/CT, including 212 confirmed cases, with an intra-CTE mortality rate of 39%; 48 admissions and 3 recoveries today while 5 intra-CTE deaths have been reported.

- **In Haut-Uélé** : the absence of standardized CTE persists in the 6 affected health zones

1.6.3. Challenges

Insufficient availability of referral resources (4 ambulances out of service in Ituri, no ambulance in 7 health zones in North Kivu) and saturation of several CTE/CT facilities, limiting rapid access to care and isolation of suspects.

1.7. Risk communication and community engagement

1.7.1. Main actions

- **In Ituri**, 75 community health workers from five health areas were briefed on CREC and provided with awareness-raising materials; 6,546 people were reached during home visits and 3,596 during educational talks.
- **In North Kivu**, 25 confirmed cases were the subject of communication actions with 111 household heads; 15 rumors were clarified and 1 community incident resolved.
- **In Haut-Uélé**, 8,602 people were reached during mass awareness campaigns in the health zones of Isiro and Wamba and 66 IEC tools were distributed (cumulative: 351).

1.7.2. Challenges

Persistent community challenges: demotivation of stakeholders due to non-payment for over two months in North Kivu, denial of Ebola virus disease and opposition to a disease prevention and control program in Isiro, repeated delays in vaccine availability, likely to affect community participation in response interventions

1.8. Logistics

1.8.1. Main actions

In Ituri : inauguration of the Kasenyi CTE with a capacity of 80 beds; the installed capacity reaches 840 beds (412 for confirmed cases, 428 for suspected cases); the expansion of the Bambu, Nizi and ISTM CTEs is requested in the face of saturation.

In North Kivu : reception and analysis of the order of PCI inputs for the Kalunguta health zone and development of the distribution plan for dignity kits received from UNFPA; continuation of the construction of the tank bases at the PoCs of Karuruma and Komba and of the triage unit of the Vuhovi General Referral Hospital (ULB Cooperation).

1.8.2. Challenges

Persistence of logistical needs to support the expansion of the response, including the completion of CTE extensions, the continued supply of essential inputs and PPE, and the availability of means of transport and evacuation.

1.9. Security 1.9.1.

Main actions

- In North Kivu, security support was provided to the EDS team for the transfer of a lifeless body from the morgue of the Beni General Referral Hospital to the Mukuliya barrier; securing the Naissance Désirable health center, in the Butsili health area, which was threatened with fire by the population.

1.9.2. Challenges

- Persistent insecurity, incidents involving political authorities, and community resistance during dignified and secure burials, limiting access for teams

Response, continuity of interventions and coverage of activities in certain affected areas: aggression against teams in burial areas in North Kivu, armed resistance preventing joint interventions in Isiro (Haut-Uélé) and 10 dignified and secure burials postponed until the following day in Ituri.

1.10. PSEA (Prevention of Sexual Exploitation and Abuse)

1.10.1. Main actions

- Continued awareness-raising activities on Prevention of Sexual Exploitation and Abuse (PSEA) in the various communities of the affected provinces.

1.10.2. The challenges

Coverage of awareness-raising activities and strengthening of mechanisms for the prevention, reporting and management of cases of sexual exploitation and abuse remains insufficient across all areas covered by the response.

KEY MESSAGES

Ebola transmission remains active, with 87 new cases and 51 deaths, including 34 fatalities.

community and 17 intra-CTE recorded in the last 24 hours, bringing the national total to 4,381 confirmed cases and 2,011 deaths.

Ituri remains the epicenter of the epidemic, concentrating 85.8% of cumulative confirmed cases and 65.5% of new cases reported in the last 24 hours and a greater expansion (28 of the 36 health zones affected).

The monitoring system has performed well over the past 24 hours, with 100% of alerts investigated and a follow-up rate of 86.7% exceeding the threshold of 85.0%.

Strengthening priority interventions (surveillance, care, IPC, logistics and community engagement) remains essential to rapidly interrupt transmission chains.

TIMELINE OF KEY EVENTS

From April 24 to May 15, 2026, the epidemic went from detection of the first suspected cases through confirmation

biological of the Ebola Bundibugyo virus, leading to the

Official declaration of the 17th outbreak of Virus Disease

Ebola in the Democratic Republic of Congo.

Between May 17 and 18, 2026, the national authorities, with

The support of partners has strengthened the response through the

declaration of a public health emergency of broad scope

national and then continental, accompanied by the

mobilization of coordination mechanisms and

support.



EBOLA PRESIDENTIAL FORCE TASK 17
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